A-A-54369
15 April 1991

COMMERCIAL ITEM DESCRIPTION

DEFIBRILLATOR AND CARDIOSCOPE

(12-LEAD, WITH RECORDER)

The General Services Administration has authorized the use of this Commercial Item Description.

This Commercial Item Description covers a defibrillator system that will respond to cardiac crisis and trauma situations in a hospital environment.

Salient characteristics:

The cardiac care system shall be a modular ECG monitor/recorder and defibrillator, which can operate as an integral system or as two separate units. Both modules shall be independently powered, have integral battery chargers and be capable of functioning from self-contained batteries and AC line power.

The system shall include a diagnostic quality ECG recorder, a heart rate meter with high and low rate alarms, an energy meter, defibrillator paddles, synchronized cardioversion capability and accessories as specified herein. Provision shall be made for out-of-the-way paddle storage.

The system shall be comprised of only two separate components, which shall securely interconnect into a physically contiguous unit without the use of tools, without the need for auxiliary cables, and in such a manner as not to cause excessive stress on the electrical connectors. The system will be required to respond to a life-threatening emergency, a testing situation or a standby role in a hospital. It may be used for monitoring beside the bed, during surgery or while transporting a patient.

AMSC N/A FSC 6515

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The system's modules either used separately or together as an integral unit shall, as a minimum, comply with all applicable sections and requirements (not addressed herein), of the following publications (current editions):

* ANSI/AAMI DF2 - American National Standard for Cardiac Defibrillator Devices.

* UL 544- Standard for Safety of Electric Medical and Dental Equipment.

* NFPA 99 - Standard for Health Care Facilities.

* American Heart Association, AHA publication titled "Recommendations for Standardization of Specifications for Instruments in Electrocardiography and Vectorcardiography."

All equipment must be capable of enduring environments the equipment is expected to experience in shipment, storage, service and use.

Atmospheric pressure	19.5 in Hg to 30.1 in Hg
Relative Humidity (R.H.)	95 percent (non-condensing)
Temperature:	
Operational	32°F to 114°F
Storage	-23°F to 149°F

All units shall be easily cleaned with available hospital cleansing solutions and adequately protected from moisture. The unit shall be of a rugged construction capable of withstanding hazards of normal use in a hospital setting such as shock and impact, frequently encountered in the hospital environment.

The total weight of the ECG Monitor/Recorder and Defibrillator, including the batteries, paddles, etc., shall not exceed forty (40) pounds.

ECG Monitor/Recorder:

1. Input. The unit shall be capable of providing an ECG through the defibrillator paddles or via a five (5) lead patient cable having a six (6) pin cable connector conforming to AAMI ECG - 5/83, Standard for ECG Connectors.

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a. The patient cable leads supplied with the monitor shall be color coded in accordance with AHA publication, "Recommendations for Standardization of Leads and of Specifications for Instruments in Electrocardiography and Vectorcardiography."

b. The patient cable shall be shielded and have a minimum length of 10 ft. Individual lead length shall be approximately 3 ft. Total cable length with patient leads should not exceed 13 ft.

2. Cardioscope display. The cardioscope display shall have the following characteristics:

a. Size: 4 in minimum diagonal, non-fade type display.

b. Sweep speed: 25 mm/second.

c. Frequency response (-3 db): No less than 0.5 Hz to 30 Hz.

d. To avoid false alarms or operator error the display must show a representation of a heart pacemaker pulse on the CRT, but not count the signal on the digital HR display.

e. ECG gain shall be continuously adjustable or discrete step mode.

f. ECG signal shall have an easily adjustable audio volume control.

3. ECG Amplifier. The ECG amplifier shall have the following characteristics:

a. Lead selection. Full 12 (I, II, III, aVR, aVL, aVF, V).

b. Frequency response. 0.05 Hz to 100 Hz.

c. Common mode rejection. 100 dB minimum with respect to chassis ground with 5k ohms imbalance at 60 Hz. 65 dB minimum with respect to isolated ground.

d. Defibrillator protection. Protected for standard LOWN or equivalent differential pulse to 400 watt seconds terminated to patient.

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e. Calibration. Momentary 1 millivolt signal to preamplifier.

f. Leakage current. The unit shall have an electrically isolated patient input circuit and leakage currents in accordance with the latest edition of ANSI/AAMI ES1, Safe Current Limits for Electromedical Apparatus.

4. Recorder. The recorder shall be an integral part of the monitor module. A stand-alone instrument requiring interconnecting cabling, or integral with the defibrillator module, shall not be acceptable. The recorder stylus position shall be adjustable and capable of being changed by the operator. The recorder display shall have the following characteristics:

a. Paper size. 1.97 in width x 100 ft, 0.04 in x 0.2 in ruled squares.

b. Paper speed. 1 in/second

c. Low warning. Visual end of roll marking shall appear on paper no less than three feet (3 ft) from the end.

d. Recorder modes. Real time and delayed by approximately 5 seconds.

e. Frequency response. 0.05 Hz to 100 Hz (real), 0.5Hz to 40 Hz (delay).

5. Heart rate meter. The heart rate meter shall have the following characteristics:

a. Rate detector: Operate with patient-generated R-wave; positive or negative polarity.

b. Rate display: 3 digit LED or equilvalent digital display.

c. Range: At least 30 to 240 beats/minute, 5 percent accuracy.

d. Alarm range: Adjustable, minimum range 30 to 240 BPM: high and low settings.

e. Update frequency: 2-5 seconds - shall not be affected by activation of freeze control, if provided.

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Violation of the alarm limits shall result in automatic recorder activation and an audible alarm tone. Recorder trace shall be for a timed sequence of approximately six (6) to twelve (12) seconds, delayed sufficiently to record the event that triggered the alarm.

Defibrillator. The defibrillator shall have the following characteristics and, as a minimum, comply with all the applicable requirements of the current edition of the American National Standard for Cardiac Defibrillator Devices, ANSI/AAMI DF2, with the exception of paragraph 3.2.1.2, Energy Range.

1. Waveform: Nominal 5 millisecond monophasic pulse duration with Edmark, LOWN, or Trapeziodal waveform is acceptable.

2. Output energy: At least 5-320 joules range continuously adjustable or in a (delivered) series of at least eight discrete steps when delivered into a 50 ohm load.

Defibrillator shall be capable of delivering 10 pulses with maximum energies into a 50 ohm resistive load within 5 minutes.

Defibrillator output shall be electrically isolated.

3. Charge time: Maximum 10 seconds, 320 joules or more.

4. Change control: Independent momentary pushbutton on front panel and one external paddle.

5. External adult paddle electrode area: At least 7.75 in^2 each.

Pediatric paddle electrode area: At least 2.33 in^2 each.

Cord Length: Approximately 10 ft.

6. Defibrillator synchronizer: Synchronizes defibrillator pulse to patient-generated R-wave.

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7. Synchronization indicator: Marker on cardioscope shall identify synchronizer trigger point. Defibrillator shall discharge within 30mS after marker. Marker shall appear on ECG recorder when in delay mode.

The instrument shall switch back to defibrillation mode after each cardioversion attempt.

Energy level/charge indicator. The defibrillator shall be supplied with a digital energy level/charge display which shall clearly indicate to the user two discrete functions:

1. One function shall digitally indicate the level of energy, in joules, either available for delivery into a 50 ohm load prior to discharge, or that has been discharged into the patient, calibrated as if discharged into a 50 ohm load. This measurement/display shall be performed at every available energy setting, accurate to within plus or minus 15 percent, or 4 joules, whichever is greater.

2. The other function shall digitally indicate, using the same display, the actual energy, in joules, when delivered into the defibrillator's built-in 50 ohm test load. This measurement/display shall be performed on at least one designed energy setting of 160 joules or above, and accurate to within plus or minus 15 percent, or 4 joules, whichever is greater.

Paddle requirements and types.

1. The defibrillator shall be provided with the following two types of paddles:

* External adult set.
* Pediatric set.

2. The following additional two types shall be available options.

* Internal adult set
* External anterior/posterior set.

3. All four types shall connect/disconnect rapidly to/from the defibrillator chassis.

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4. External adult and pediatric paddles shall be capable of ECG pick-up through the paddle surface. Discharge control of the external paddles shall be by pushbuttons located on each paddle which must be simultaneously activated to fire the unit.

5. Internal paddles shall be charged and discharged from the defibrillator. Maximum selectable delivered energy shall be 50 joules when internal paddles are connected.

6. Discharge control of the external anterior/posterior paddles shall, at a minimum, be at least on the anterior paddle.

7. All paddles shall be capable of being used as ECG elecctrodes for observation on the ECG monitor/recorder.

Electrical Characteristics.

1. The ECG monitor/recorder and defibrillator shall operate over the full tolerance range of one or more of the following line voltage and frequency combinations:

115 VAC plus or minus 10 percent, 50 plus or minus 2 Hz
115 VAC plus or minus 10 percent, 60 plus or minus 2 Hz
230 VAC plus or minus 10 percent, 50 plus or minus 2 Hz
230 VAC plus or minus 10 percent, 60 plus or minus 2 Hz

Dual voltage and hertz on all units are desirable but not mandatory.

2. Change over devices, switches, circuits, or transformers if used for the purpose of voltage or frequency conversion, shall be clearly marked and identified in the appropriate documentation which accompanies the unit. Mechanical or electrical adjustments to effect the conversion shall not require the use of specialized tools or skills.

3. AC line power operation shall provide all functions specified herein, independent of the internal batteries' charge state, while simultaneously charging these batteries. When disconnected from AC line power, the system shall be capable of providing all functions specified herein from internal battery power.

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4. The battery power source shall have the following characteristics:

a. Battery type: Rechargeable

b. Battery capacity: 4 hours of continuous cardioscope monitoring or 1 hour of continuous recording (or any linear combination) at 25°C with a HR of 60 BPM and a 1.5 cm cardioscope display.

Nominal thirty (30) discharges of maximum energy at 77°F.

c. Battery meter(s). Shall indicate approximate voltage level of batteries.

d. Battery charge indicator(s): Shall illuminate when batteries are charging.

e. Battery charge time: 16 hours maximum to full charge for each depleted battery.

5. The battery power source shall be installed in the equipment, and in a condition which optimizes shelf-life and minimizes potential damage for the particular battery type.

6. In addition to the requirements specified the contractor shall include specific technical data on battery care and maintenance.

Accessories. Each ECG monitor/recorder and defibrillator shall be supplied with all equipment and accessories required so that each unit comprises a complete operational system. It shall include as a minimum, but not be limited to, the following:

1. One each - Electrode accessory kit to include the following items:

* Four each - Adult limb electrodes.
* One each - Welsh suction cup, 30 mm.
* One each - Welsh suction cup, 15 mm.
* Four each - Adult limb strap, 40 cm.
* One each - Patient cable, 5 lead, 6-pin connector with pin ends for use wilth metal electrodes.

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2. One pair - Pediatric paddles.
3. Two each - 5-lead patient cable with female snap type contact.
4. One each - 18 mm suction cup electrode.
5. One each - Stylus, spare.
6. One each - Stylus insertion tool.
7. One each - ECG ruler.
8. Three each - Rolls, ECG paper.
9. Twelve each - Pre-jelled electrodes.
10. One pair - External adult paddles.
11. One tube - Electrode jelly.
12. One each - Accessory bag.

13. Where a monitor/recorder and defibrillator are designed to be separated, and a patient can be monitored via the monitor which is not directly connected to the defibrillator, the contractor in his bid/proposal shall specify any additional components required to facilitate a remote synchronized cardioversion. Items shall be separately identified and priced.

The unit shall be of a rugged construction capable of withstanding hazards of normal use in a hospital setting.

Operational and Service Data Manuals shall be furnished as follows:

a. Information Content:

The contractor shall furnish manuals, handbooks and/or brochures containing complete operation, installation and service/maintenance instructions (including pictures of illustrations, as necessary) with complete schematics and wiring diagrams. All manuals, handbooks, and/or brochures shall be written in the English language and all schematics and wiring diagrams shall use American electrical and electronics symbols. The manuals shall include electrical data and connection diagrams for all applicable utilities. The instructions shall also contain a complete list of all replaceable parts showing part number, name and quantity required. All service notes, service memos, etc., pertaining to the equipment and issued subsequent to the printing of the original instructions shall also be included. These instructions shall be, as a minimum, that furnished to service engineers (or servicemen) that normally install and service the equipment for the company or distributor. When the system being procured includes, or will operate ancillary equipment, the service data shall include complete instructions and drawings which show interfacing of all system components.

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b. Quantities, distribution and schedules:

Two (2) complete sets of the above information shall be packaged with each unit of the required equipment. Where multiple quantities of the same unit are to be delivered to the same ultimate consignee, then only a total of 2 service manuals need to be supplied to each consignee. However, 2 operation manuals still need to be supplied with each unit.

Workmanship. The defibrillator/ECG monitor shall be free from defects which detract from its appearance or impair its serviceability.

Unit. Each (EA). One defibrillator/ECG monitor, as specified, constitutes one unit.

Quality Assurance Provisions.

Responsibility for inspection. Unless otherwise specified in the contract or purchase order, the contractor is responsible for the performance of all inspection requirements as specified herein. Except as otherwise specified in the contract or purchase order, the contractor may use his own or any facilities suitable for the performance of the inspection requirements specified herein, unless disapproved by the Government. The Government reserves the right to perform any of the inspections set forth in the specification where such inspections are deemed necessary to assure supplies and services conform to prescribed requirements.

Records. Records of examinations and tests performed by or for the contractor shall be maintained by the contractor and made available to the Government upon the Government's request, at any time, or from time to time, during the performance of the contract and for a period of three years after delivery of the supplies to which such records relate.

Inspection. Inspection, as used herein, is defined as both examination (such as visual or auditory investigation without the use of special laboratory appliances or procedures) and testing (determination by technical means of physical and chemical properties) of the item.

Sampling for examination. Sampling for examination shall be conducted in accordance with MIL STD-105 with an AQL of 1.0 (percent defective) and an inspection level of II. The unit of product for examination purposes shall be one defibrillator and cardioscope.

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Sampling for tests. Sampling for tests, including dimensional test, shall be conducted in accordance with MIL STD-105 with an AQL of 1.5 (percent defective) and an inspection level of S-2. The unit of product for test purposes shall be one defibrillator and cardioscope.

Tests. Tests shall be conducted to determine compliance with specification requirements. Where feasible, the same sample shall be used for the determination of two or more test characteristics. Tests shall include, but shall not be limited to, the following:

Defibrillator leakage test. Chassis leakage - Leakage current measured between any exposed conductive surface and ground shall not exceed 100 uA.

Paddle Leakage - Leakage current measured between an external paddle electrode and ground shall not exceed 100 uA. Leakage current measured between an internal paddle electrode and ground shall not exceed 50 uA.

Conditions - Leakage current measurements will be made under all of the following conditions.

* Power on and off.

* Defibrillator in both charged and uncharged state.

ECG monitor/recorder leakage current test. Chassis leakage - Leakage current measured between any exposed conductive surface and ground shall not exceed 100 uA.

Lead Leakage (source) - Leakage current measured between each lead and ground, all leads and ground, and between any two leads shall not exceed 10 uA.

Condition - Leakage current measurement will be performed with power on and off.

Lead Leakage (Sink) - Sink current measured between each lead at a potential of 120 V and ground shall not exceed 20 uA. This test shall be performed under the conditions listed above except the ground remains intact.

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Strain relief test. Gradually apply a 35 lb. pull in a direction that would be the most likely to pull the cord from the enclosure and maintain for one minute. The strain relief is not acceptable if there is such movement of the cord as to indicate that stress is being transmitted to terminals, splices or interior wiring.

Air clearance spacing test. Measurements shall be made using an appropriate measuring instrument, to verify isolated patient connected circuitry is spaced at least 1/8 inch from non-isolated circuitry.

Defibrillator paddle insulation test. Apply aluminum foil to the entire gripping surface of one paddle with sufficient pressure to make it conform to the surface.

With the paddles connected to the defibrillator, connect the high potential lead of a 60 Hz (or 50 Hz, if preferred) high potential test source to the aluminum foil. Connect low-potential or ground lead to the paddle electrode. Slowly adjust the peak voltage amplitude of the test source to equal 1.5 times the maximum voltage occurring on the defibrillator energy stored capacitor. Maintain the voltage for 1 minute.

Repeat this test with the low-potential lead connected to the other paddle electrode.

There shall be no voltage breakdown, and the 60 Hz leakage current shall not exceed 250 uA peak (177 uA rms); if a 50 Hz test source is used, the current shall not exceed 210 uA peak (148 uA rms).

ECG monitor/recorder sweep/chart speed test. Measure the monitor sweep speed and the recorder paper speed by applying a signal of known frequency to the patient cable. Monitor and recorder chart sweep speed shall be within plus or minus 10 percent and plus or minus 5 percent of 25 mm/sec respectively.

ECG amplifier calibration test. The accuracy of the 1 mV internal calibration pulse shall be measured by comparing the amplitude of this pulse to a pulse applied to the patient cable from a pulse generator. It shall be accurate to plus or minus 15 percent.

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ECG amplifier CMRR test. Common mode rejection is measured by comparing the amplitude of a 1 mV differential signal applied to the monitor to the amplitude of a higher voltage sinusoidal signal of 60 Hz applied between the junction of the leads and ground. Measured common mode rejection shall be at least 100 db with respect to chassis ground with 5 k ohms imbalance between leads.

Defibrillator energy accuracy test. Connect the defibrillator paddles to a defibrillator analyzer equipped with a 50 ohm resistive load. Charge the defibrillator to a pre-selected energy level, then discharge into the analyzer. Repeat for each available energy level setting. Measure and record data. Measured energy must be within plus or minus 15 percent (or 4 joules, whichever is greater) of the selected energy.

Defibrillator charge time test. Using a stopwatch, measure the time between activation of the charge control and full charge indication for the first and last of 10 sequential charges and discharges all at 320 joules or above. This time shall not exceed 10 seconds.

Synchronized discharge test. Connect a signal generator to the ECG monitor. Measure the time between the leading edge of square pulse and the leading edge of the defibrillator's synchronized discharge pulse. This time can be displayed automatically on a defibrillator analyzer if so equipped, or measured directly from an oscilloscope, and shall be within 30 milliseconds.

ECG monitor and recorder frequency response test. Apply a 1 mV peak to peak differential sinusoidal signal to the patient cable, and vary the frequency. The bandwidth is defined by the upper and lower frequencies at which the trace deflection amplitude decreases to 0.707 times the maximum value. This bandwidth shall be 0.5 Hz to 30 Hz on the cardioscope and 0.05 to 100 Hz on the recorder.

Humidity test. Place the unit in test chamber such that leakage current and delivered energy accuracy test can be performed external to the chamber.

Adjust the temperature of the chamber to 82°F.

Permit sufficient time for the temperature "inside the unit" to stabilize prior to proceeding.

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After temperature stabilization, adjust the relative humidity to 95 percent and permit sufficient time for humidity stabilization.

After 48 hours of conditioning, source and sink leakage current test are to be performed. Record results on leakage current summary charts.

Perform the energy accuracy test as specified in the Defibrillator energy accuracy test paragraph above.

The unit shall be in compliance, if leakage current and energy accuracy are within the specified limits.

Battery capacity test. Verify 4.0 hours of continuous cardioscope monitoring or 1.0 hour of continuous recording of a heart rate of 60 BPM. Defibrillator battery will be tested by verifying thirty (30)-320 joules discharges at 77°F.

Defibrillator discharge pulse rate test. Apply the paddle electrodes to a defibrillator tester and select an energy level of at least 320 joules. Simultaneously initiate the charge and a timer.

Immediately upon completion of the charge, discharge the defibrillator.

The defibrillator shall be capable of delivering 10 pulses at 320 joules, into a 50 ohm resistive load within 5 minutes, at 0°C.

Defibrillator overload protection test. Overload protection shall comply with those requirements specified in ANSI/AAMI EC 13-1983 American National Standard for Cardiac Monitors, Heart Rate Meters and Alarms.

Electrical characteristics test. Operate monitor/recorder and defibrillator units over the full tolerance range of the following line voltage and frequency combinations:

115 VAC plus or minus 10 percent, 50 plus or minus 2Hz
115 VAC plus or minus 10 percent, 60 plus or minus 2Hz
230 VAC plus or minus 10 percent, 50 plus or minus 2Hz
230 VAC plus or minus 10 percent, 60 plus or minus 2Hz

Metric products. Products manufactured to metric dimensions will be considered on an equal basis with those manufactured using inch-pound units, providing they fall within the tolerances specified using conversion tables contained in the latest revision of Federal Standard 376, and all other requirements of this document are met.

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If a product is manufactured to metric dimensions and those dimensions exceed the tolerances specified in the inch/pound units, a request should be made to the contracting officer to determine if the product is acceptable.

The contracting officer has the option of accepting or rejecting the product.

Contractor certification. The contractor shall certify that the product offered meets the salient characteristics of this description and conforms to the producers' own drawings, specifications, standards, and quality assurance practices. The Government reserves the right to require proof of such conformance prior to first delivery and thereafter as may be otherwise provided for under the provisions of the contract.

Regulatory requirements.

Federal Food, Drug and Cosmetic Act. If the product covered by this document has been determined by the U.S. Food and Drug Administration to be under its jurisdiction, the offeror/contractor shall comply, and be responsible for compliance by its subcontractors/suppliers, with the requirements of the Federal Food, Drug and Cosmetic Act, as amended, and regulations promulgated thereunder. In addition, the offeror/contractor shall comply, and be responsible for compliance by its subcontractors/suppliers, with the requirements of all other applicable Federal, State, and local statutes, ordinances, and regulations.

Recovered materials. The offeror/contractor is encouraged to use recovered material in accordance with Federal Acquisition Regulation Subpart 23.4 to the maximum extent practical.

Preservation, packaging, packing, labeling, and marking. Unless otherwise specified, preservation, packaging, and packing shall be to a degree of protection to preclude damage to containers and/or contents thereof under normal shipping conditions, handling, etc., involving shipment from the supply source to the receiving activity, plus reshipment from receiving activity, and shall conform to applicable carrier's rules and regulations. Intermediate and exterior package quantities and labeling and marking shall be as specified in the contract and/or order.

Ordering data. Intermediate/exterior package quantities, labeling, and marking must be specified in the contract and/or order.

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MILITARY INTERESTS:

Custodians:

Army - MD
Navy - MS
Air Force - 03

PREPARING ACTIVITY:

DoD - MB

Agent:

DLA-DM

CIVIL AGENCY COORDINATING ACTIVITIES:

VA-OSS
USPHS
FDA-MPQAS

Project No. 6515-5009

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I RECOMMEND A CHANGE:	1. DOCUMENT NUMBER	2. DOCUMENT DATE (YYMMDD)
	A-A-54369	15 April 1991

3. DOCUMENT TITLE
DEFIBRILLATOR AND CARDIOSCOPE (12-Lead, with Recorder)

4. NATURE OF CHANGE (Identify paragraph number and include proposed rewrite, if possible. Attach extra sheets as needed.)

5. REASON FOR RECOMMENDATION

6. SUBMITTER

a. NAME (Last, First, Middle Initial)	b. ORGANIZATION
c. ADDRESS (Include Zip Code)	d. TELEPHONE (Include Area Code) (1) Commercial (2) AUTOVON (If applicable) / 7. DATE SUBMITTED (YYMMDD)

8. PREPARING ACTIVITY

a. NAME	b. TELEPHONE (Include Area Code) (1) Commercial / (2) AUTOVON
DEFENSE PERSONNEL SUPPORT CENTER ATTN: DPSC-MSTE	AC 215-737-2870 / AV 444-2870
c. ADDRESS (Include Zip Code) 2800 SO. 20TH STREET PHILADELPHIA, PA 19101-8419	IF YOU DO NOT RECEIVE A REPLY WITHIN 45 DAYS, CONTACT: Defense Quality and Standardization Office 5203 Leesburg Pike Suite 1403, Falls Church, VA 22041-3466 Telephone (703) 756-2340 AUTOVON 289-2340

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